

Post-Injectable Timing Windows

How long after Diana injects before you may treat

DRAFT — not policy until signed. Conservative defaults from standard aesthetic practice. Diana, RN and the medical director own the final numbers. Until signed, the answer in the room is "let me check."

AFTER AN INJECTABLE → BEFORE YOUR SERVICE

AFTER...	NO TOUCH	MASSAGE · LYMPHATIC · SCULPTING	DERMAPLANE	NEEDLING / CHANNELING	RF · HEAT · GLO2	WAXING
Neurotoxin Botox · Daxxify	24 h	7 days	7 days	14 days	14 days	7 days
Dermal filler	24 h	14 days	14 days	14 days	28 days	14 days
PRF — injected	24 h	14 days	14 days	14 days	14 days	14 days

Why. Neurotoxin stays **mobile** for the first days — pressure migrates it, and migration into the levator is a **drooping eyelid for three months**. Filler needs ~**two weeks to settle**; pressure, needling, or heat before that moves it or breaks it down. **Heat accelerates HA breakdown** — that's why RF gets the longest window.

BEFORE DIANA INJECTS → AFTER YOUR SERVICE

AFTER THE CLIENT HAD...	WAIT BEFORE TOX · FILLER · PRF INJECTION
Microneedling · microchanneling · PRF fibrin veil	14 days
BioRePeel · TCA · medium peel	14 days
Light peel · enzyme · hydrodermabrasion	7 days
Glo2Facial · basic facial · dermaplaning	48 h
Waxing over the injection site	48 h

THE FOUR OVERRIDES THAT BEAT THE CALENDAR

- Bruising, swelling, or tenderness still visible → do not treat**, no matter how many days have passed. The calendar is a floor, not a permission slip.
- Never same-day.** No injectable + needling, peel, or energy device the same day. Not as a package, not as a favor, not because she drove from Naples.
- When in doubt, ask Diana.** She has the chart, the product, the volume, the placement. You have a guess.
- An outside injector's instructions win.** Get them in writing — or apply the **longest** window on this card.

ASK EVERY FACIAL CLIENT — IT IS NOT ON THE INTAKE FORM
"Any Botox, Daxxify, or filler recently? How recently?"

Front desk: ask at **booking**, not check-in. Flag the file with the injectable date. If the service falls inside a window, **book her past it** — never "we'll check when she gets here." And whenever she wants both: **book the facial BEFORE the tox.**

☐ Diana Morrison, RN _____ date _____ ☐ Medical Director _____ date _____

Pregnancy & Breastfeeding

Hard stops · the product tray · positioning

DRAFT — not policy until signed by the medical director. Conservative defaults. Her OB always has the final say.

ASK DIRECTLY — AT BOOKING AND AT INTAKE. NEVER GUESS FROM APPEARANCE.
"Are you pregnant, breastfeeding, or trying to conceive?"

HARD STOP — DO NOT PERFORM

SERVICE	PROVIDER	WHY
All microneedling / microchanneling	Amber	Infection risk · anesthetic absorption · no safety data
PRF — topical or injected	Amber / Diana	Blood draw · no safety data
Neurotoxin · Dermal filler	Diana	Contraindicated
IV therapy	Diana	Fluid / additive load — OB clearance
GLP-1 — Semaglutide / Tirzepatide	Diana	Contraindicated — includes "trying to conceive"
BioRePeel · TCA · medium peels	Amber	Systemic absorption
RF · heat · energy · electrical modalities	Amber / Brandy	Not established as safe

Never treat a pregnant client "just gently" through a hard-stop service. The hard stops are hard. · Breastfeeding: every hard stop above still applies.

THE PRODUCT TRAY

NEVER

- **RETINOIDS** — any Vitamin A derivative. Retinol, retinal, tretinoin, Retin-A, adapalene, tazarotene. **High-dose Vitamin A is linked to birth defects. This is the most important line on this card.**
- **HYDROQUINONE** — 30–40% systemic absorption. The highest of anything on our shelf.
- **High-% salicylic acid · BHA peels.** Low-strength is arguably fine — **house rule: avoid entirely.** It removes the judgment call from a busy room.
- TCA · BioRePeel · medium/deep peels
- High-% benzoyl peroxide
- Essential-oil-heavy / aromatherapy blends
- Formaldehyde releasers · chemical depilatories

SAFE — REACH FOR THESE

- Hyaluronic acid · glycerin · ceramides · peptides
- **Niacinamide · azelaic acid**
- Vitamin C (L-ascorbic)
- **Bakuchiol** — the pregnancy-safe retinol alternative. **Tell her about this.** She wants to know there's an option.
- PHAs · **low-strength lactic / glycolic / mandelic**
- Enzyme exfoliants — papain, bromelain
- **Mineral SPF** — zinc oxide / titanium dioxide

Permitted services: gentle facials · hydration · enzyme exfoliation · hydrodermabrasion (gentle) · dermaplaning (lighter) · massage with positioning · waxing (more sensitive, more bruising — say so first) · mineral makeup. **Permanent jewelry: defer.**

AFTER ~20 WEEKS — DO NOT LAY HER FLAT ON HER BACK

She can faint on your table. The uterus compresses the **vena cava**, her blood pressure drops, and she gets dizzy and nauseated — supine hypotensive syndrome. **Incline the bed, or wedge her tilted to the left.** Bolster under the knees. **Check in verbally throughout.** Dizziness, nausea, or shortness of breath → **sit her up immediately.** First-trimester nausea makes strong scents intolerable — **keep the room neutral.** She may need the bathroom mid-service: **build the buffer in and say so up front** so she doesn't feel awkward asking.

THE MELASMA CONVERSATION — SHE WILL ASK

Pregnancy hormones drive melasma hard. **Mineral SPF is the answer, and it's the honest one.** No hydroquinone, no aggressive peels, no heat — not now. *"We'll build the real plan once you're done breastfeeding. Between now and then, sun protection is the single highest-value thing you can do — it's what keeps this from getting harder to treat later."* **That conversation keeps the client. It doesn't lose her.**
Never market a service as **"safe for pregnancy"** — say **"we adjust the treatment and the products, and your OB has the final say."** Never tell a client to stop a prescription; that is her prescriber's call.

☐ Medical Director _____ date _____

☐ Posted in both rooms _____ date _____